# Bevacizumab 10 · colorectal

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Bevacizumab component: Day 1 every 14 days

**Active medicines:** Bevacizumab

**Emetogenic risk:** Minimal (bevacizumab component only)

No routine antiemetic prophylaxis for the bevacizumab component. The prescribed fluoropyrimidine-based chemotherapy requires its own antiemetic and supportive-care plan; minimal risk here does not classify the whole combination.

### Additional phase-specific support

- Premedication is not usually required unless there has been previous hypersensitivity; the treating team specifies any individual plan.
- Review blood pressure, urine protein, bleeding, thrombosis, wounds and planned procedures before dosing.
- Provide a prescribed diarrhoea plan appropriate to the chemotherapy partner; bevacizumab does not define a universal loperamide schedule.
- Urgent assessment for significant bleeding, severe abdominal pain, chest pain, sudden breathlessness, severe headache, confusion, seizure or visual disturbance.
- Use the separate chemotherapy schedule and support prescription. No lung six-cycle limit or ovarian course cap applies to this colorectal context.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00212 v7 · p5 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-05 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required